



Pathway

Giant Cell Arteritis (GCA)

Scope

Site	Service/Department/Unit	Disciplines
SCGH	Hospital-Wide	Medical

Entry criteria

1. Clinical suspicion of GCA
2. Patient greater than 50 years old

Relative exclusion criteria

Note GCA is a clinical diagnosis with no clear-cut exclusion criteria. It is unlikely where the following is present. Discussion is always welcome.

- ☐ ENT /ocular or peri-ocular inflammation – refer to Ophthalmology/ENT as appropriate
- ☐ Kidney, skin, peripheral nervous system involvement
- ☐ Lung infiltrates
- ☐ Lymphadenopathy
- ☐ Stiff neck
- ☐ Gangrene or ulceration
- ☐ Headache >6 months duration

Differentials explored before entry to pathway and considered unlikely:

- Meningo/encephalitis
- Cervicogenic headache
- Raised Intracranial pressure/Space Occupying Lesion
- Infection – Dental/Urinary/Respiratory/Other

REQUEST: ESR, CRP, FBC, BSL, Urine MCS

Calculate score based on revised ACT criteria 2016

MUST HAVE 1 POINT FROM DOMAIN I	
DOMAIN I	
New onset localized headache	1
Sudden Onset of Visual Disturbances	1
Polymyalgia Rheumatica	2
Jaw Claudication	1
Abnormal temporal artery examination	2
DOMAIN II	
Unexplained fever or anaemia	1
ESR ≥ 50 mmHr	1
Compatible Pathology (biopsy of TAB)	2

0-2	GCA unlikely Seek alternate diagnosis
3-4	GCA possible Contact Rheumatology Registrar Mon-Friday 08:30-16:30 Contact Rheumatology Consultant after hours Contact Ultrasound Department for urgent Temporal Arteritis Ultrasound (during working hours) <u>If Amaurosis Fugax/painless monocular visual loss:</u> Admit patient. Urgent discussion with Rheumatology registrar or Consultant as appropriate. Same day Ophthalmology review.
≥ 5	GCA likely Contact Rheumatology Registrar Mon-Friday 08:30-16:30 Contact Rheumatology Consultant after hours Contact Ultrasound Department for urgent Temporal Arteritis Ultrasound (during working hours) <u>If Amaurosis Fugax/painless monocular visual loss:</u> Admit patient. Urgent discussion with Rheumatology registrar or Consultant as appropriate. Same day Ophthalmology review.
	If requires admission: Admit under Rheumatology Mon-Thurs 08:30-16:30, Friday 08:30-12:00pm Admit MAU other times, for Rheumatology TOC next working day If <u>Amaurosis Fugax or painless visual loss:</u> Urgent Ophthalmology review.

For modified ACR greater than 3, after Rheumatology review:

When CRP greater than 24.3 or ESR greater than 50mmHr, **must** have diagnostic workup within 72 hours.

For modified ACR less than 3, after Rheumatology review:

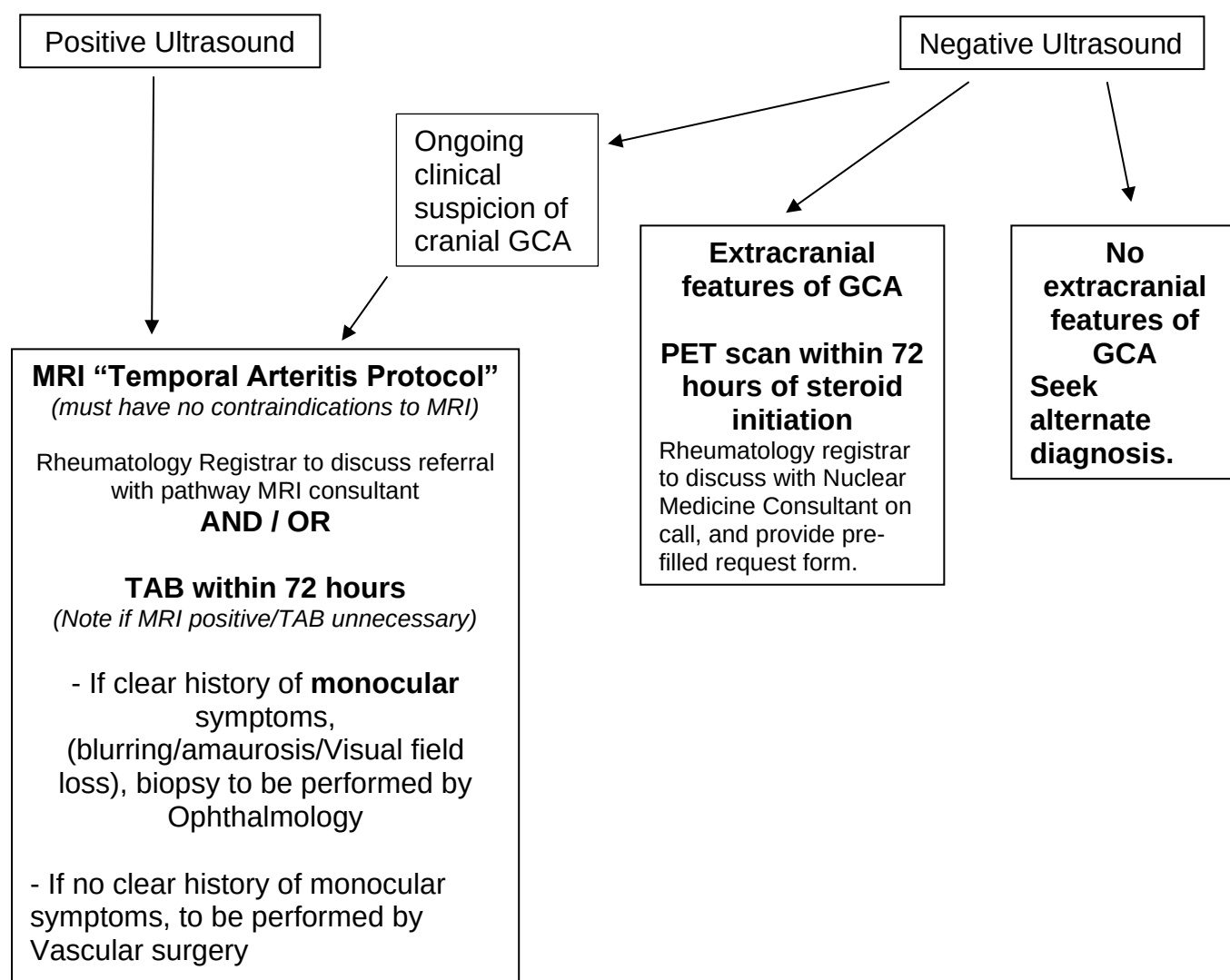
If CRP less than 24.5 AND ESR less than 50mmHr, may need diagnostic workup within 72 hours

Based on 2016 Modified ACR classification criteria for GCA.

If clinical suspicion high, despite low score, then discussion with Rheumatology is advised.

After Rheumatologist Discussion:

Treatment initiated 40-60mg oral Prednisolone or equivalent
 Urgent Temporal Artery Ultrasound - ideally within 24 hours
 Radiology department (Not available weekends)
 On request write "Temporal Arteritis (GCA) pathway – urgent study"



If diagnosed with Temporal Arteritis and >65 years age:

1. Occupational Therapy referral for MOCA on ward – Page 3033
2. Refer for outpatient comprehensive geriatric assessment via frailty clinic. in the body of the referral write *gca study - FRAC - Dr Bernard * as the first line.

Authorisation & Version Control

Policy Sponsor	Area Director Clinical Services				
Policy Contact	Head of Department Rheumatology				
Date First Issued:	09/09/2022	Last Reviewed:		Review Date:	09/09/2024
Version No.	1	Reference #:	HG 46		
Approved by:	Area Director Clinical Services			Date:	09/09/2022
Endorsed by:	Area Director Clinical Services, Executive sponsor of the Policy and Guideline Committee			Date:	09/09/2022
Acknowledgments:	Krista Makin, Head of Department Rheumatology, Debbie Olsson-White Rheumatologist, Maxine Isbel Rheumatology Registrar				
Standards Applicable	Insert relevant NSQHS Standards (Version 2): 				
Aboriginal Health ISD Number:					

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

To provide feedback on the Pathway please email PolicyOfficer.SCGOPHCG@health.wa.gov.au